

1. Administrative & Study Identification

Full Study Title: A Study to Evaluate Efficacy of Remibrutinib Compared to Dupilumab at Early Timepoints in Adults With Chronic Spontaneous Urticaria Inadequately Controlled by Second Generation H1-antihistamines **Short Title/Acronym:** RECLAIM
Protocol Number: CLOU064AUS02 **NCT Number:** NCT06868212 **Sponsor Name:** Novartis Pharmaceuticals (Company: Novartis)
Investigational Product: Remibrutinib and Dupilumab **Phase of Development:** Phase IIb (Phase 3) **Trial Status:** RECRUITING (Currently recruiting participants) **Medical Monitor:** Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate and compare the efficacy of remibrutinib versus dupilumab at early timepoints (specifically at 4 weeks) when administered as an add-on treatment in adult patients with moderate to severe chronic spontaneous urticaria (CSU). **Secondary Objectives:** To assess absolute change from baseline in Urticaria Activity Score (UAS7) at week 1; absolute changes in Itch Severity Score (ISS7) and Hives Severity Score (HSS7) at week 4; achievement of well-controlled disease (UAS7 $\leq$ 6) at weeks 2 and 4; complete disease control (UAS7 = 0) at week 4; and overall safety assessments.

2.2 Background & Rationale To address the gap in fast-acting, effective treatments for patients with moderate-to-severe Chronic Spontaneous Urticaria (CSU) who remain symptomatic despite standard therapy with second-generation H1-antihistamines (sgH1-AHs). Remibrutinib is a highly selective oral Bruton's tyrosine kinase (BTK) inhibitor (a targeted class that includes the reference drug ibrutinib [Trade Name: Imbruvica, ATC Code: L01EL01, EMA URL: https://www.ema.europa.eu/en/documents/product-information/imbruvica-epar-product-information_en.pdf]), and dupilumab is a subcutaneous interleukin-4 receptor- α monoclonal antibody. This study aims to evaluate the early onset of efficacy of these targeted therapies in a head-to-head, double-dummy trial design.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator and Placebo-controlled **Blinding:** Double-blind, Double-dummy **Randomization:** 1:1 (Stratified by prior exposure to anti-IgE biologics)

3.2 Planned Enrollment & Duration Targeted Enrollment: 400
 evaluable patients **Number of Sites:** Multi-center (US-based)
Estimated Study Start: [DD-MMM-YYYY] (Recruiting as of Q1 2026)
Estimated Primary Completion: [DD-MMM-YYYY]

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adults ≥ 18 years of age at the time of signing the informed consent. 2. CSU duration for ≥ 6 months prior to screening. 3. Diagnosis of CSU inadequately controlled by sgH1-AH at the time of randomization, defined as the presence of itch and hives for ≥ 6 consecutive weeks prior to screening with UAS7 score ≥ 16 , ISS7 score ≥ 6 , and HSS7 score ≥ 6 . 4. Documentation of hives within 3 months before randomization.

4.2 Exclusion Criteria 1. Patients with chronic inducible urticaria or other skin diseases with symptoms of urticaria, angioedema, or chronic itching. 2. Prior use of remibrutinib, BTK inhibitors, or dupilumab. 3. Clinically significant cardiovascular (e.g., NYHA Class III/IV, recent myocardial infarction), neurological, psychiatric, pulmonary, renal, hepatic disorder, or immunodeficiency. 4. Requirement for dual anti-platelet medication or anticoagulant medication (warfarin or NOACs).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: - Arm 1: Remibrutinib 25 mg twice daily (b.i.d.) by mouth + matching placebo solution for injection. - Arm 2: Dupilumab 600 mg loading dose followed by 300 mg every 2 weeks + remibrutinib matching placebo pills. **Route of Administration:** Oral (p.o.) and Subcutaneous (s.c.) **Duration of Treatment:** 12-week core double-blind treatment period, followed by an optional 12-week open-label extension (OLE) period.

5.2 Concomitant & Prohibited Therapy Permitted: Stable, standard label dose of a sgH1-AH ("background therapy"). Rescue therapy of the same sgH1-AH is allowed up to a total maximum daily dose of 4 tablets/day. **Prohibited:** Other anti-IgE biologics, alternative BTK inhibitors, dual anti-platelet therapy, and anticoagulants.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to -7	Informed Consent, Medical History, Establish Eligibility
Baseline	Day 0	Randomization (1:1), First Dose of Study Drug/Placebo
Interim	Weeks 1, 2, 4, 12	Safety Labs, Efficacy Assessment (UAS7, ISS7, HSS7 tracking)
End of Study	Week 24	Final Vital Signs, Exit Interview, IP Accountability (End of OLE or Safety Follow-up)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Absolute change from baseline in weekly Urticaria Activity Score (UAS7) at Week 4. **Measurement Method:** Urticaria Patient Daily Diary (UPDD) symptom tracking.

7.2 Secondary & Exploratory Endpoints 1. Absolute change from baseline in UAS7 at week 1. 2. Absolute change from baseline in weekly Itch Severity Score (ISS7) and Hives Severity Score (HSS7) at week 4. 3. Achievement of well-controlled disease (UAS7 ≤ 6) and complete disease control (UAS7 = 0) at weeks 2 and 4.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Patient diary tracking, onsite assessments, and safety follow-up phone calls at Weeks 16 and 24 (for non-OLE participants). **Serious Adverse Events (SAEs):** Standard reporting timeline (typically 24 hours). **Data Monitoring Committee (DMC):** Internal safety review committee established by the Sponsor.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy analyses. Safety Set for AE/SAE measurements. **Sample Size Justification:** $N = 400$ participants, adequately powered to detect a statistically significant difference in early efficacy (UAS7 score reduction) between remibrutinib and dupilumab. **Handling of Missing Data:** Multiple Imputation (MI) and Last Observation Carried Forward (LOCF) for daily diary score drop-offs.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits by Clinical Research Associates (CRAs) to ensure protocol adherence. **Audit Trail:** Detailed tracking of eCRF entries and data clarification forms (DCFs) with a full audit history.

11. Study Identifiers & Governance

Posting ID: 159497182413785272 **Primary ID:** CLOU064AUS02 **Registry Number:** NCT06868212 **Phase Requirement:** PHASE3 (Trial must be in PHASE3) **Sponsor/Lead Organization:** Novartis Pharmaceuticals **Study Title:** RECLAIM **Official Regulatory Title:** A Study to Evaluate Efficacy of Remibrutinib Compared to Dupilumab at Early Timepoints in Adults With Chronic Spontaneous Urticaria Inadequately Controlled by Second Generation H1-antihistamines

12. Clinical Framework (CSU Specific)

Primary Condition: Chronic Spontaneous Urticaria (CSU) **Semantic Type:** Disease or Syndrome **Condition Definition:** Urticaria characterized by spontaneously recurring hives for 6 weeks or longer. [PMID:25807072] **Ontology Codes:** SNOMED ID: 302162004 | MeSH Code: D000080223 | HPO Code: HP:0410133 **Diagnosis Threshold:** UAS7 ≥ 16 , ISS7 ≥ 6 , and HSS7 ≥ 6 despite sgH1-AH therapy **Medical Methodology:** Targeted therapy added to background second generation H1-antihistamines **Optimization Target:** Well-controlled disease (UAS7 ≤ 6) or complete control (UAS7 = 0)

13. Study Procedures & Methodology

Management Pathway: Double-dummy blinded addition of oral vs. subcutaneous targeted therapy. **Education Protocol (HCP):** Clinical site training for double-dummy administration and severity score calculations. **Education Protocol (Patient):** Onsite training for Urticaria Patient Daily Diary (UPDD) completion. **Quality Audit Mechanism:** Daily diary compliance tracking and routine monitoring visits.

14. Observation & Follow-Up Schedule

Total Duration: Up to 28 weeks (4-week screening + 12-week core double-blind + 12-week OLE/safety follow-up). **Onsite Visit Cadence:** Baseline, Weeks 1, 2, 4, 12, and 24. **Remote Monitoring Frequency:** Safety follow-up phone calls at Weeks 16 and 24 for those not entering the OLE. **Checklist Review Interval:** Continuous tracking via Patient Daily Diary.

15. Sign-off & Approval

Role	Name	Signature	Date				
Principal Investigator	_____	_____	[DD-MMM-YYYY]			Clinical	
Operations Lead	_____	_____	[DD-MMM-YYYY]			Lead Statistician	
	_____	_____	[DD-MMM-YYYY]				